

Trodelvy (Sacituzumab govitecan-hziy)

Original Effective Date: 10/1/2025

Revision Date(s): 10/1/2025

Review Date: 10/1/2025

Policy type: Medical Necessity

Line of Business: All GEHA Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Indication Specific Criteria

Unresectable Locally Advanced or Metastatic Breast Cancer

- The member is 18 years of age or older; **AND**
- The member has an ECOG performance status of 0-1¹; **AND**
- The member has HER2 negative (IHC 0, IHC 1+ or IHC 2+/ISH-) and hormone receptor (HR) positive disease; **AND**
 - The member has progressed on prior endocrine and CDK 4/6 inhibitor therapy; **AND**
 - The member has progressed on at least two prior lines of systemic chemotherapy (at least one of these prior lines in the metastatic setting); **AND**
 - The member is not a candidate for fam-trastuzumab deruxtecan-nxki (Enhertu); **OR**
- The member has triple negative breast cancer; **AND**
 - The member has progressed on or after at least two prior lines of systemic therapy (at least one of these prior lines in the metastatic setting)

Limitations

- The treatment of members with urothelial cancer with Trodelvy is not recommended.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**

¹ 0 to 1 - ECOG score of 2 or 3 will be considered on a case-by-case basis

- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
 - Clinical Pharmacology (Strong For); **OR**
 - Wolters Kluwer Lexi-Drugs® (Level A); **OR**
- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
 - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;
 - **Meeting abstracts and case reports are excluded from consideration;**
- Non-standard protocols may be approved based on unique clinical circumstances;
- Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network® (NCCN), or indication specific peer-reviewed literature;
- In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing:

Drug Name	HCP Code	Description
Trodelvy	J9317	Injection, sacituzumab govitecan-hziy, 2.5 mg

References

1. Trodelvy. [package insert]. Gilead Sciences, Inc. Foster City, CA. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/761115s059lbl.pdf
2. Trodelvy. NCCN Drugs & Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/
3. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed

September 5, 2025. To view the most recent and complete version of the guideline, go online to NCCN.org.

4. Powles, T. et al. Sacituzumab govitecan in advanced urothelial carcinoma: TROPiCS-04, a phase III randomized trial. *Annals of Oncology*, Volume 36, Issue 5, 561 – 571.

Disclaimer

Drug Coverage Policies are developed as needed, reviewed and updated at least annually, and are subject to change. Other policies and coverage determination guidelines may apply. Federal and state regulatory requirements and member specific benefit plan documents, if applicable, must be reviewed prior to utilizing this Drug Coverage Policy. This Drug Coverage Policy is for informational purposes only and does not constitute medical advice nor dictate how providers should practice medicine. This policy should not be reproduced, stored in a retrieval system, or altered from its original form without written permission from OncoHealth, Inc.

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Date	Updates
10/1/2025	New med nec policy presented and approved by OH P&T
10/23/25	Policy adopted by GEHA; document updated for universal accessibility.